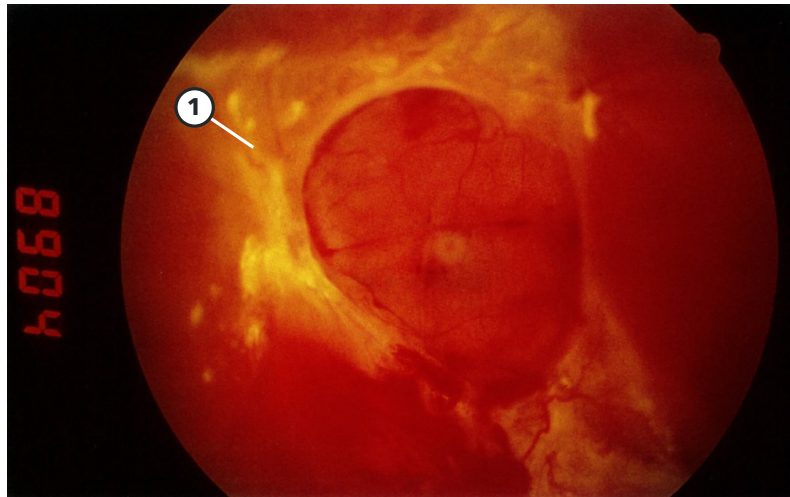


PDR: traction, blood and surgical thresholds

Clinical M3-M5 | Focused Type B notes

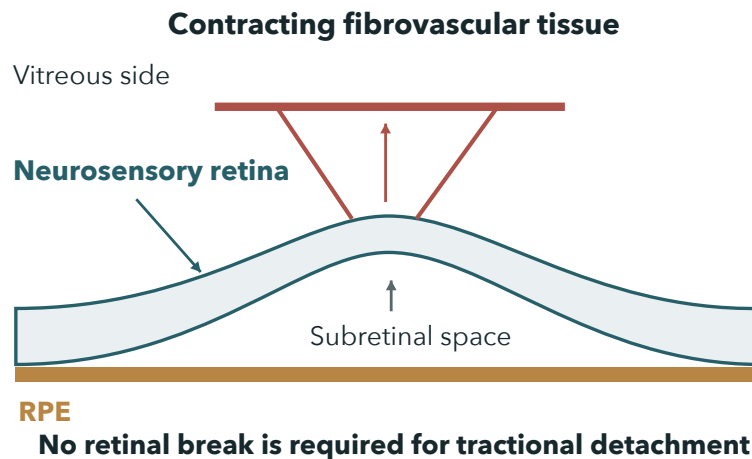
Mechanical traction needs a mechanical solution

Fibrovascular tissue can lift the retina



1: fibrovascular bands. Established traction can elevate the retina; OCT and examination determine whether the fovea is threatened. Supplied M3 Anki, media 116.

Tractional detachment



Original cross-section: contracting tissue pulls the neurosensory retina away from the RPE, even without a full-thickness break.

Surgical threshold

Offer vitreoretinal surgical assessment for PDR detachment involving or threatening the macula. An anti-VEGF injection or more laser alone cannot reliably release the established fibrovascular traction.

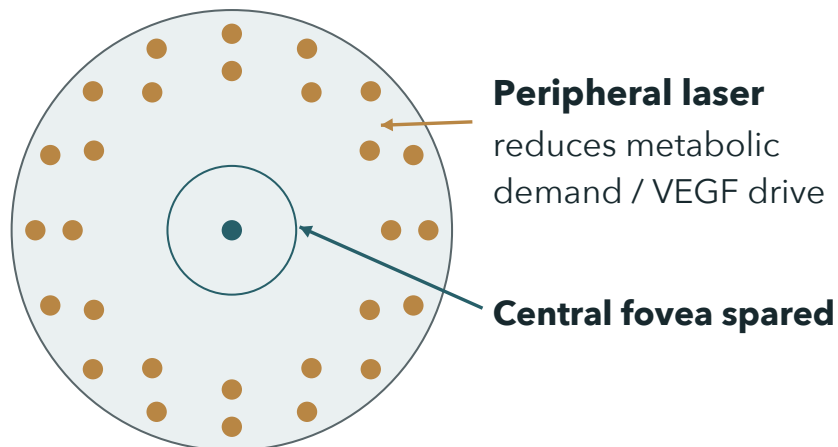
References NICE NG 242: Diabetic retinopathy management | AAO: Retinal detachment

Persistent blood and the PRP trade-off

When to discuss vitrectomy

Situation	Why surgery enters the plan
Non-clearing PDR vitreous haemorrhage	Consider vitrectomy when blood has not cleared within three months under NICE guidance.
Recurrent blood or active traction despite completed PRP	Assess structural threat and suitability for surgery.
Macula-threatening/involving detachment	Release traction and protect central retinal structure.

PRP protects sight through a peripheral trade-off



Consent: peripheral field / night vision may worsen

Original schematic. Peripheral laser reduces ischaemic drive; field/night-vision effects should be discussed during consent.

Review after treatment still matters

PRP does not guarantee that every complication stops. Follow acuity, macular OCT, neovascular activity, traction and haemorrhage; persistent loss needs an explanation and a revised plan.